



Bill Number: S.B. 1678

Dunn SUBSTITUTE Floor Amendment

Reference to: printed bill

Amendment drafted by: Leg. Council

FLOOR AMENDMENT EXPLANATION

1. Requires each assisted living center or home that contacts an emergency responder on behalf of a resident to:
 - a) note in the resident's record that the assisted living center or home provided the written document of patient information required in accordance with statute to the emergency responder; and
 - b) obtain written or electronic acknowledgement from the emergency responder that the emergency responder received the written document.
2. Removes the requirement for an employee of a health care institution, group home or behavioral-supported group home who provides direct care to a patient or resident to complete a form prescribed by the Department of Health Services (DHS), which may be electronic, for each patient or resident under the employee's care to update the patient's or resident's information before the end of the employee's workday or shift and another employee assumes the care of the patient or resident.
3. Requires DHS, if the department receives a complaint from an emergency medical care technician or peace officer alleging that a health care institution failed to provide a patient's or resident's do-not-resuscitate order or other advance directive, if on file with the health care institution, after requesting emergency medical services, to determine whether the health care institution:
 - a) is in compliance with the patient information disclosure requirements in accordance with statute;
 - b) has policies and procedures established, documented and implemented that cover advance directives, including do-not-resuscitate orders; and
 - c) complied with the policies and procedures in the situation that was basis for the complaint.
4. Requires DHS, if the department receives a complaint from an emergency medical care technician or peace officer alleging that a group home or behavioral-supported group home failed to provide a resident's do-not-resuscitate order or other advance directive, if on file with the group home or behavioral-supported group home, after requesting emergency medical services, to determine whether the group home or behavioral-supported group home:
 - a) has policies and procedures established, documented and implemented that cover advance directives, including do-not-resuscitate orders; and
 - b) complied with the policies and procedures in the situation that was basis for the complaint.
5. States that the compliance requirements for advance directives do not require a patient or resident to have any advance directive, including a do-not-resuscitate order, on file with a health care institution, group home or behavioral-supported group home unless otherwise specified by law.
6. Defines *emergency medical technician*.

Amendment explanation prepared by Hayden Darst

3/2/2026

DUNN SUBSTITUTE FLOOR AMENDMENT
SENATE AMENDMENTS TO S.B. 1678

I move the following SUBSTITUTE amendment to the REGULATORY AFFAIRS AND GOVERNMENT EFFICIENCY Committee Amendment to
SENATE BILL 1678 (Reference to printed bill)

Amendment instruction key:
[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.
[Green underlining in brackets] indicates text added to new session law or text restoring existing law.
[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.
[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.
<<Green carets>> indicate a section added to the bill.
<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 <<Section 1. Section 36-420.04, Arizona Revised Statutes, is
3 amended to read:

4 36-420.04. Assisted living centers and homes; emergency
5 responders; resident information; hospitals;
6 discharge planning; patient screenings;
7 discharge documents

8 A. An assisted living center or assisted living home that contacts
9 an emergency responder on behalf of a resident shall provide to the
10 emergency responder a written document that includes all of the following:

11 1. The reason or reasons the emergency responder was requested on
12 behalf of the resident.

13 2. Whether the resident receives medication services and, if the
14 resident has provided this information to the assisted living center or
15 assisted living home, a list of all the resident's prescription and
16 over-the-counter medications, their dosages and how frequently they are
17 administered.

18 3. The name, address and telephone number of the resident's current
19 pharmacy.

20 4. A list of any known allergies to any medications, additives,
21 preservatives or materials [Strike] [.. SUCH AS] latex or adhesive.

22 5. The name and contact information for the resident's primary care
23 physician and power of attorney or authorized representative.

24 6. Basic information about the resident's physical and mental
25 conditions and basic medical history, such as having diabetes or a

1 pacemaker or experiencing frequent falls or cardiovascular and
2 cerebrovascular events, as well as dates of recent episodes, if known.

3 7. The point-of-contact information for the assisted living center
4 or assisted living home, including the telephone number [AND], if
5 available, [A] cell phone number and email address. A point of contact
6 must be available to respond to questions regarding the information
7 provided twenty-four hours a day, seven days a week.

8 8. A copy of the resident's health insurance portability and
9 accountability act release authorizing a receiving hospital to communicate
10 with the assisted living center or assisted living home to plan for the
11 resident's discharge. This paragraph does not preclude a resident from
12 revoking the resident's health insurance portability and accountability
13 act release authorization.

14 9. A copy of the resident's advance directives, if any, on file at
15 the assisted living center or assisted living home. This paragraph does
16 not preclude a resident from revoking or modifying the resident's advance
17 directives.

18 [B. EACH ASSISTED LIVING CENTER OR ASSISTED LIVING HOME THAT
19 CONTACTS AN EMERGENCY RESPONDER ON BEHALF OF A RESIDENT SHALL NOTE IN THE
20 RESIDENT'S RECORD THAT THE ASSISTED LIVING CENTER OR ASSISTED LIVING HOME
21 PROVIDED THE DOCUMENT REQUIRED BY SUBSECTION A OF THIS SECTION TO THE
22 EMERGENCY RESPONDER AND SHALL OBTAIN WRITTEN OR ELECTRONIC ACKNOWLEDGMENT
23 FROM THE EMERGENCY RESPONDER THAT THE EMERGENCY RESPONDER RECEIVED THE
24 DOCUMENT.]

25 [B.] [C.] The assisted living center or assisted living home must
26 notify the resident's authorized representative that the resident was
27 transported to a hospital and provide the name and location of the
28 hospital.

29 [C.] [D.] Each assisted living center and assisted living home
30 must maintain a standardized form for each resident that includes the
31 information prescribed in subsection A of this section, except for the
32 information prescribed in subsection A, paragraph 1 of this section, which
33 shall be provided at the time the emergency responder is contacted. Each
34 assisted living center and assisted living home shall periodically update
35 this form for each resident as necessary.

36 [D.] [E.] An assisted living center or assisted living home shall
37 maintain a copy of the document provided to the emergency responder and
38 documentation of the actions required by subsection [B.] [C] of this
39 section for a period of two years after the date of the emergency.

40 [E.] [F.] If the emergency responder transports the resident to a
41 hospital, the emergency responder shall provide a copy of the written
42 document required by subsection A of this section to the receiving
43 hospital.

44 [F.] [G.] To protect the health and safety of patients being
45 transferred by a hospital to an assisted living center or assisted living
46 home, a discharging hospital shall coordinate with the health care
47 institution from which the patient has been transferred or the health care

1 institution to which the patient will be discharged and shall provide a
2 written discharge plan for each inpatient. The discharge plan shall:

3 1. Be prepared by appropriate staff.

4 2. Include point-of-contact information for the discharging
5 hospital, including a telephone number and, if available, an email
6 address. The hospital's point of contact shall monitor the telephone and,
7 if available, the email address provided and shall promptly respond to any
8 inquiry. For forty-eight hours after discharge, the discharging
9 hospital's designated point of contact must be available to respond to the
10 receiving assisted living center or assisted living home to assist in
11 returning or admitting the patient to the facility and to clarify any
12 needed information in the discharge plan. Hospitals shall have a
13 qualified hospital representative respond to discharge plan questions
14 received from the assisted living center or assisted living home.

15 3. Document the patient's discharge evaluation and provide an
16 assessment of the patient's medical or health conditions, including:

17 (a) Any documented pressure injuries or ulcers, the location on the
18 body and the assessed stage level.

19 (b) Cognitive or physical conditions or impairments.

20 (c) The patient's weight-bearing status.

21 (d) Specified dietary requirements, if applicable.

22 (e) Whether the patient requires continuous medical services or
23 continuous or intermittent nursing services or restraints.

24 (f) Whether the patient requires specialized medical equipment or
25 home health services and a copy of the hospital's orders for that
26 equipment or those services.

27 (g) Follow-up health care services and other services recommended
28 for the patient.

29 4. Include, if applicable, a copy of any prescription that was
30 transmitted to the patient's current pharmacy as designated in subsection
31 A of this section or the medication summary or medication instructions.
32 The copy of the prescription or medication summary or medication
33 instructions shall include the patient's name, medication administration
34 instructions and the signature of the prescriber or a record that the
35 prescription was electronically signed by the prescriber.

36 5. Document that the hospital notified the receiving assisted
37 living center or assisted living home of any new device orders for the
38 patient.

39 6. Document that the discharging hospital notified the patient's
40 authorized representative that the patient was discharged and provided the
41 name, location and contact information of the receiving facility.

42 ~~[G.]~~ [H.] The discharging hospital shall contact the assisted
43 living center or assisted living home from which the patient entered the
44 hospital, after initial inpatient assessment, to discuss an evaluation of
45 the patient's likely postdischarge health care needs. The hospital shall
46 reevaluate and discuss with the assisted living center or assisted living
47 home from which the patient entered the hospital the patient's condition,

1 as appropriate, to identify changes to the patient's condition that may
2 impact the patient's postdischarge health care needs, including
3 recommendations, if any, by the hospital to transfer the patient to a
4 different facility other than the facility from which the patient entered
5 the hospital to address the patient's higher care needs. The discharging
6 hospital shall provide an opportunity for a patient screening before
7 discharge by the assisted living center or assisted living home from which
8 the patient entered the hospital or to which the patient is being
9 referred. The assisted living center or assisted living home shall
10 determine through a screening and a review of or discussion about medical
11 records whether the patient's postdischarge care needs, including
12 additional ordered services, are within the facility's scope of services.
13 After receiving notification from the discharging hospital, the assisted
14 living center or assisted living home shall perform the screening
15 promptly. For on-site screenings, the staff of the assisted living center
16 or assisted living home may be required to follow standard hospital
17 security and identification requirements. If the assisted living center
18 or assisted living home from which the patient entered the hospital can no
19 longer meet the patient's needs, the facility shall provide documentation
20 to the hospital demonstrating the reasons why the facility cannot accept
21 the patient back to the facility.

22 ~~[H.]~~ [I.] For patients being transferred from a hospital to an
23 assisted living center or assisted living home who have not previously
24 been admitted to the assisted living center or assisted living home, the
25 hospital shall coordinate with the assisted living center or assisted
26 living home to allow time to expeditiously obtain documentation from a
27 physician, registered nurse practitioner, registered nurse or physician
28 assistant coordinating the level of care needed in the assisted living
29 center or assisted living home.

30 ~~[F.]~~ [J.] For emergency department or observation patients, the
31 hospital shall provide the assisted living center or assisted living home
32 from which the patient entered the hospital with a point of contact,
33 including a telephone number and, if available, an email address. The
34 hospital's point of contact shall monitor the telephone and, if available,
35 the email address provided and promptly respond to any inquiry. For
36 forty-eight hours after discharge, the discharging hospital's designated
37 point of contact must be available to respond to the receiving assisted
38 living center or assisted living home to assist in returning or admitting
39 the patient to the assisted living center or assisted living home and to
40 clarify any needed information in the discharge plan. Hospitals shall
41 have a qualified hospital representative respond to discharge plan
42 questions received from the assisted living center or assisted living
43 home. The hospital shall also provide the patient's assisted living
44 center or assisted living home with any new prescription orders as
45 prescribed in subsection ~~[F.]~~ [G.], paragraph 4 of this section, a record
46 of medications administered and any identified follow-up care services
47 recommended for the patient. The hospital shall provide the additional

1 information outlined in subsection ~~[F]~~ [G], paragraph 3 of this section
2 if the hospital determines it is applicable. An assisted living center or
3 assisted living home from which the patient entered the hospital shall
4 readmit the patient if the assisted living center or assisted living home
5 can meet the care needs for the patient.

6 ~~[J.]~~ [K.] Each hospital shall develop a discharge document that
7 encompasses the information outlined in subsection ~~[F]~~ [G] of this
8 section for inpatient discharges. The discharge document shall be
9 provided to the assisted living center or assisted living home to which
10 the patient is being discharged.>>

11 Section 2. Title 36, chapter 4, article 1, Arizona Revised
12 Statutes, is amended by adding section 36-420.06, to read:

13 36-420.06. Patients and residents; advance directives;
14 do-not-resuscitate orders; policies and
15 procedures; definitions

16 A. ~~[AN EMPLOYEE OF] [IF THE DEPARTMENT RECEIVES A COMPLAINT FROM AN~~
17 ~~EMERGENCY MEDICAL CARE TECHNICIAN OR PEACE OFFICER ALLEGING THAT] A HEALTH~~
18 ~~CARE INSTITUTION [WHO PROVIDES DIRECT CARE TO A PATIENT OR RESIDENT SHALL~~
19 ~~COMPLETE A FORM AS PRESCRIBED BY THE DEPARTMENT, WHICH MAY BE ELECTRONIC,~~
20 ~~FOR EACH PATIENT OR RESIDENT UNDER THE EMPLOYEE'S CARE TO UPDATE THE]~~
21 ~~[FAILED TO PROVIDE A] PATIENT'S OR RESIDENT'S [INFORMATION BEFORE THE END~~
22 ~~OF THE EMPLOYEE'S WORKDAY OR SHIFT AND ANOTHER EMPLOYEE ASSUMES THE CARE~~
23 ~~OF THE PATIENT OR RESIDENT] [DO-NOT-RESUSCITATE ORDER OR OTHER ADVANCE~~
24 ~~DIRECTIVE, IF ON FILE WITH THE HEALTH CARE INSTITUTION, AFTER REQUESTING~~
25 ~~EMERGENCY MEDICAL SERVICES, THE DEPARTMENT SHALL DETERMINE WHETHER THE~~
26 ~~HEALTH CARE INSTITUTION:~~

27 1. IS IN COMPLIANCE WITH SECTION 36-420.04, IF APPLICABLE.
28 2. HAS POLICIES AND PROCEDURES ESTABLISHED, DOCUMENTED AND
29 IMPLEMENTED THAT COVER ADVANCE DIRECTIVES, INCLUDING DO-NOT-RESUSCITATE
30 ORDERS.
31 3. COMPLIED WITH THE POLICIES AND PROCEDURES IN THE SITUATION THAT
32 WAS THE BASIS FOR THE COMPLAINT].

33 [THE FORM SHALL INCLUDE AT LEAST THE FOLLOWING:

34 1. THE PATIENT'S OR RESIDENT'S NAME AND DATE OF BIRTH.
35 2. THE PATIENT'S OR RESIDENT'S ADVANCE DIRECTIVES, INCLUDING A DO
36 NOT RESUSCITATE ORDER OR PROVIDER ORDERS FOR LIFE-SUSTAINING TREATMENT,
37 AND A RELEASE ALLOWING THE HEALTH CARE INSTITUTION TO SHARE THE PATIENT'S
38 OR RESIDENT'S PROTECTED HEALTH INFORMATION WITH A THIRD PARTY.
39 3. THE PATIENT'S OR RESIDENT'S CURRENT VITAL SIGNS, INCLUDING BLOOD
40 PRESSURE, HEART RATE, RESPIRATORY RATE, OXYGEN SATURATION AND TEMPERATURE,
41 AND ANY RECENT LABORATORY RESULTS, SUCH AS BLOOD GLUCOSE.
42 4. A LIST OF THE PATIENT'S OR RESIDENT'S MEDICATIONS AND ANY
43 ALLERGIES.
44 5. THE NAME OF THE PATIENT'S OR RESIDENT'S PRIMARY CARE PROVIDER,
45 EMERGENCY CONTACT OR POWER OF ATTORNEY.
46 6. THE HEALTH CARE INSTITUTION'S POINT OF CONTACT.]

1 [B. THIS SECTION DOES NOT REQUIRE A PATIENT OR RESIDENT TO HAVE ANY
2 ADVANCE DIRECTIVE, INCLUDING A DO-NOT-RESUSCITATE ORDER, ON FILE WITH A
3 HEALTH CARE INSTITUTION UNLESS OTHERWISE REQUIRED BY LAW.]

4 [B.] [C.] FOR THE PURPOSES OF THIS SECTION[.] [.]

5 [1. "EMERGENCY MEDICAL CARE TECHNICIAN" HAS THE SAME MEANING
6 PRESCRIBED IN SECTION 36-2201.

7 2.] "HEALTH CARE INSTITUTION" MEANS AN ASSISTED LIVING CENTER, AN
8 ASSISTED LIVING HOME, AN ASSISTED LIVING FACILITY, HOSPICE, A
9 NURSING-SUPPORTED GROUP HOME, A NURSING CARE INSTITUTION OR A RESIDENTIAL
10 CARE INSTITUTION THAT IS LICENSED PURSUANT TO THIS CHAPTER.

11 Sec. 3. Title 36, chapter 5.1, article 1, Arizona Revised Statutes,
12 is amended by adding section 36-576, to read:

13 36-576. Residents; advance directives; do-not-resuscitate
14 orders; policies and procedures; rules;
15 definition

16 A. [AN EMPLOYEE OF] [IF THE DEPARTMENT RECEIVES A COMPLAINT FROM AN
17 EMERGENCY MEDICAL CARE TECHNICIAN OR PEACE OFFICER ALLEGING THAT] A GROUP
18 HOME OR BEHAVIORAL-SUPPORTED GROUP HOME [WHO PROVIDES DIRECT CARE TO A
19 RESIDENT SHALL COMPLETE A FORM AS PRESCRIBED BY THE DEPARTMENT, WHICH MAY
20 BE ELECTRONIC, FOR EACH RESIDENT UNDER THE EMPLOYEE'S CARE TO UPDATE THE]
21 [FAILED TO PROVIDE A] RESIDENT'S [INFORMATION BEFORE THE END OF THE
22 EMPLOYEE'S WORKDAY OR SHIFT AND ANOTHER EMPLOYEE ASSUMES THE CARE OF THE
23 RESIDENT] [DO-NOT-RESUSCITATE ORDER OR OTHER ADVANCE DIRECTIVE, IF ON FILE
24 WITH THE GROUP HOME OR BEHAVIORAL-SUPPORTED GROUP HOME, AFTER REQUESTING
25 EMERGENCY MEDICAL SERVICES, THE DEPARTMENT SHALL CONDUCT AN INVESTIGATION
26 TO DETERMINE WHETHER THE GROUP HOME OR BEHAVIORAL-SUPPORTED GROUP HOME:

27 1. HAS POLICIES AND PROCEDURES ESTABLISHED, DOCUMENTED AND
28 IMPLEMENTED THAT COVER ADVANCE DIRECTIVES, INCLUDING DO-NOT-RESUSCITATE
29 ORDERS.

30 2. COMPLIED WITH THE POLICIES AND PROCEDURES IN THE SITUATION THAT
31 WAS THE BASIS FOR THE COMPLAINT].

32 [THE FORM SHALL INCLUDE AT LEAST THE FOLLOWING:

33 1. THE RESIDENT'S NAME AND DATE OF BIRTH.

34 2. THE RESIDENT'S ADVANCE DIRECTIVES, INCLUDING A DO NOT
35 RESUSCITATE ORDER OR PROVIDER ORDERS FOR LIFE-SUSTAINING TREATMENT, AND A
36 RELEASE ALLOWING THE GROUP HOME OR BEHAVIORAL-SUPPORTED GROUP HOME TO
37 SHARE THE RESIDENT'S PROTECTED HEALTH INFORMATION WITH A THIRD PARTY.

38 3. THE RESIDENT'S CURRENT VITAL SIGNS, INCLUDING BLOOD PRESSURE,
39 HEART RATE, RESPIRATORY RATE, OXYGEN SATURATION AND TEMPERATURE, AND ANY
40 RECENT LABORATORY RESULTS, SUCH AS BLOOD GLUCOSE.

41 4. A LIST OF THE RESIDENT'S MEDICATIONS AND ANY ALLERGIES.

42 5. THE NAME OF THE RESIDENT'S PRIMARY CARE PROVIDER, EMERGENCY
43 CONTACT OR POWER OF ATTORNEY.

44 6. THE GROUP HOME'S OR BEHAVIORAL-SUPPORTED GROUP HOME'S POINT OF
45 CONTACT.]

1 [B. THIS SECTION DOES NOT REQUIRE A RESIDENT TO HAVE ANY ADVANCE
2 DIRECTIVE, INCLUDING A DO-NOT-RESUSCITATE ORDER, ON FILE WITH A GROUP HOME
3 OR BEHAVIORAL-SUPPORTED GROUP HOME UNLESS OTHERWISE REQUIRED BY LAW.]

4 [B.] [C.] THE DEPARTMENT SHALL ADOPT RULES TO ENSURE COMPLIANCE
5 WITH THE REQUIREMENTS OF THIS SECTION.

6 [D. FOR THE PURPOSES OF THIS SECTION, "EMERGENCY MEDICAL CARE
7 TECHNICIAN" HAS THE SAME MEANING PRESCRIBED IN SECTION 36-2201.]

8 Sec. 4. Vulnerable adult system study committee; membership;
9 duties; report; delayed repeal

10 A. The vulnerable adult system study committee is established
11 consisting of the following members:

12 1. Two members of the house of representatives who are appointed by
13 the speaker of the house of representatives and who are members of
14 different political parties. The speaker of the house of representatives
15 shall designate one of these members to serve as cochairperson of the
16 committee.

17 2. Two members of the senate who are appointed by the president of
18 the senate and who are members of different political parties. The
19 president of the senate shall designate one of these members to serve as
20 cochairperson of the committee.

21 3. The director of the department of health services or the
22 director's designee.

23 4. The director of the department of economic security or the
24 director's designee.

25 5. The director of the Arizona health care cost containment system
26 or the director's designee.

27 6. One member who represents the governor's office and who is
28 appointed by the governor.

29 7. One member who represents the attorney general's office and who
30 is appointed by the attorney general.

31 8. The department of economic security's long-term care ombudsman
32 or the ombudsman's designee.

33 9. One member who currently serves as a public fiduciary and who is
34 appointed by the governor.

35 10. Two members who are employed by a local law enforcement agency
36 or by a statewide organization that represents law enforcement and who are
37 appointed by the governor.

38 11. The executive director of the Navajo area agency on aging or
39 the executive director's designee.

40 12. The executive director of the inter tribal council of Arizona
41 or the executive director's designee.

42 13. Two members who represent assisted living facilities and who
43 are appointed by the speaker of the house of representatives.

44 14. Two members who represent nursing care institutions and who are
45 appointed by the governor.

1 15. One member who represents a statewide organization that
2 advocates for elderly vulnerable adults and who is appointed by the
3 president of the senate.

4 16. One member who represents a statewide organization that
5 advocates on behalf of persons affected by Alzheimer's disease and who is
6 appointed by the president of the senate.

7 17. One member who represents a statewide association that
8 advocates on behalf of persons who provide services to persons with
9 developmental disabilities and who is appointed by the speaker of the
10 house of representatives.

11 18. One health care professional who is appointed by the speaker of
12 the house of representatives and who both:

13 (a) Is licensed pursuant to title 32, Arizona Revised Statutes.

14 (b) Provides health care services to elderly vulnerable adults.

15 19. One member who is on the governor's advisory council on aging
16 and who is appointed by the governor.

17 20. One member who represents a statewide association representing
18 firefighters in this state and who is appointed by the president of the
19 senate.

20 B. The vulnerable adult system study committee shall:

21 1. Develop and implement a coordinated vulnerable adult services
22 delivery system that ensures the health and safety of vulnerable adults.

23 2. Recommend best practices relating to responding to and
24 investigating complaints.

25 3. Research best practices related to adult protective services at
26 the state, municipality and community levels.

27 4. Research and make recommendations on how the vulnerable adult
28 system can ensure that vulnerable adults receive services they require
29 after the vulnerable adult system completes its investigatory duties,
30 including assigning a specific agency with the responsibility to provide
31 or coordinate case management.

32 5. Research and identify common statewide outcomes.

33 6. Identify best practices for data collection and data sharing by
34 various entities involved in providing vulnerable adult services.

35 7. Review and recommend changes to the statutes and rules that
36 govern vulnerable adult services.

37 C. The cochairpersons may designate work groups to research, study
38 and make recommendations to the study committee. At least two work groups
39 shall be established to separately address the needs of persons with
40 developmental disabilities and persons who are elderly or who have a
41 physical disability, including the training requirements for persons who
42 are caring for these populations.

43 D. Once the study committee establishes a strategic direction, the
44 department of economic security, in conjunction with the work groups,
45 shall develop an action plan for implementation.

1 E. Public members of the study committee are eligible to receive
2 reimbursement of expenses pursuant to title 38, chapter 4, article 2,
3 Arizona Revised Statutes.

4 F. On or before October 1, 2026 and each October 1 thereafter, the
5 study committee shall submit a report of its findings and recommendations
6 to the governor, the president of the senate and the speaker of the house
7 of representatives and shall provide a copy of this report to the
8 secretary of state.

9 G. This section is repealed from and after December 31, 2028.

10 Enroll and engross to conform

11 Amend title to conform

TIMOTHY "TIM" DUNN

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